

Staffordshire and Stoke-on-Trent Community Dermatology Referral Form
e-RS Speciality-Dermatology Service Clinic Type-All Service Type-Triage Service

Patient Details					
Name	Given Name Surname			D.O.B	Date of Birth
Address	Home Full Address (single line)			Gender	Gender(full)
				Age	Age
Tel No. (home)	Patient Home Telephone			NHS No.	NHS Number
Mobile No.	Patient Mobile Telephone				
Date of Referral	Short date letter merged	Main Language	Main Language	Interpreter Required	Yes ☐ No ☐

Referrer Details			
Address	Organisation Name Organisation Full Address (single line)		
Tel No.	Organisation Telephone Number	Practice Code	Organisation National Practice Code
Registered GP	Registered GP Full Name	Referring GP	

Clinical Information – Clinical Questions & Relevant Information

Criteria for Low and High-risk BCC
Definition of a low-risk BCC is made after excluding the following:
- Patients who are aged 24 years or younger, immunosuppressed or have Gorlin's syndrome - Lesions > 1cm below clavicle, on the face, or located over important underlying anatomical structures (for example, major vessels or nerves) or in an area where primary surgical closure may be difficult (for example, digits or front of shin) or - in an area where excision may lead to a poor cosmetic result. - Lesions that are morphoeic, infiltrative or basosquamous in appearance or have poorly defined margins

Procedure or Examination Required	
<u>ROUTINE – UP TO 18 WEEKS</u>	<u>URGENT (to consider) UP TO 7 DAYS</u>
Rashes of diagnostic uncertainty ☐	Severe Acne ☐
Moderate Acne ☐	High Risk BCC ☐
Inflammatory disorder ☐	Scarring Alopecia ☐
Moderate Eczema or Psoriasis ☐	Recurrent Infection ☐
Keloid Scars ☐	Rapid growth of lesion ☐
Urticaria ☐	Bleeding Lesion ☐
Solar Lentigo ☐	Severe Eczema or Psoriasis ☐
Non-Scarring Nail dystrophy ☐	Cutaneous Horn ☐
Pyogenic granulomata ☐	Bowens disease ☐
Dermatofibromata ☐	Severe Actinic Keratosis ☐
Benign Naevi ☐	Phototherapy – Subsequent Course Only ☐
Low Risk BCC ☐	Facial Lesions ☐
Conditions of Genitalia ☐	

Paediatric Surgical Procedures ☐

Course of Antibiotics required for Analgesia ☐

Medical Status

Is the Patient Pregnant? Yes ☐ No ☐ Is the Patient HIV Positive? Yes ☐ No ☐

Is the Patient Asthmatic? Yes ☐ No ☐ Patient had/susceptible to MRSA? Yes ☐ No ☐

Is Patient on antiplatelet drugs? Yes ☐ No ☐

Infection Risk Yes ☐ No ☐ If yes please specify

Known Allergies (These allergies are automatically populated from the clinical system. If there are any other allergies that are not recorded, then please add using the box below)

Allergies

Other Allergies
Special Needs *please tick*

Sight ☐ Hearing ☐ Speech ☐ Language ☐ Mental ☐ Physical ☐ BILCS Fax Booking Form Sent ☐

Signed (for GP Practice/Surgery)

For Office Use Only

Date Referral Sent			
Date referral received by HealthHarmonie		Date of appointment	
Date report sent to GP			

Signed (for HealthHarmonie)

HealthHarmonie Key Contacts

health.harmonie@nhs.net - Referrals can be sent to this dedicated email
hh.gpqueries@nhs.net - GP queries can be directed to this dedicated email
hh.results@nhs.net - Reports queries or reissuing of reports can be sent to this dedicated email
hh.governance@nhs.net - Feedback and governance related questions can be sent to this dedicated email

Patient & GP Service Information

 GP and patient resources can be found and requested through our website www.healthharmonie.com
Criteria for Referral

Diagnosis	Recommended Treatments / Guidelines – cks.nice.org.uk <i>Have Guidelines been followed?</i>	Tick
Acne	1. Topical Retinoid +/- Benzoyl Peroxide (Use Azelaic acid if poorly tolerated) for 3 months 2. <i>Plus</i> Oral Antibiotic – e.g. Lymecycline one capsule daily or Doxycycline 100mg daily for 3 months (Erythromycin 500mg b.d. as alternative) 3. <i>Plus</i> combined Oral Contraceptive Pill e.g. Cilest, Femodene (Reserve Dianette for severe acne only, 6 months maximum, risk VTE) <i>Refer if scarring, significant social or psychological disturbance</i>	☐ ☐ ☐
Rosacea	1. Metronidazole 0.75% cream (gel may sting) b.d. for 8 weeks (if responding continue nightly for 6 months) 2. Doxycycline MR 40 mg daily for 3 months (Erythromycin 500mg b.d. as alternative)	☐ ☐
Psoriasis	1. Emollient b.d. using 500 gram every 2 weeks 2. <i>Plus</i> Potent topical corticosteroid e.g. Betamethasone 0.1% ointment / cream once daily for 4 – 8 weeks 3. <i>Plus</i> Vitamin D analogue once daily for 4 – 8 weeks <i>Refer if >10% Body Surface Area, significant social or psychological disturbance</i>	☐ ☐ ☐
Eczema	1. Emollient b.d. using 500 gram every 2 weeks 2. <i>Plus</i> Potent topical corticosteroid e.g. Betamethasone 0.1% ointment / cream once daily for 4 weeks to body 3. <i>Plus</i> Eumovate ointment daily to face for 5 days maximum 4. Oral Antibiotics if infected – Flucloxacillin 500mg q.d.s. for 2 weeks 5. Consider oral Prednisolone 30mg daily for one week <i>Refer if >1 or 2 flares per month, facial eczema, possible Allergic Contact Dermatitis, significant social or psychological disturbance</i>	☐ ☐ ☐ ☐ ☐
Hair loss	1. Alopecia Areata – potent topical corticosteroid e.g. Betamethasone scalp application for 3 months 2. Female androgenetic alopecia - topical Minoxidil 2% (self-funded only) Check FBC, TFTS, serum Iron & Ferritin – Attach Results	☐ ☐
Fungal Nail Infection	Clippings for Mycology - Attach Results 1. Superficial infection - Amorolfine 5% nail lacquer for 9-12 months 2. Dermatophyte infection - Terbinafine 250 mg once a day; 6 weeks - 3 months for fingernails, and for 3-6 months for toenails 3. Candida Nail Infection - Pulsed Itraconazole: 200 mg twice a day for 1 week, with subsequent courses repeated after a further 21 days. Fingernails – 2 pulses ; toenails – 3 pulses	☐ ☐ ☐
Pruritus	Check FBC, ESR, U&Es, LFTs, TFTs, serum Iron & Ferritin, fasting Glucose (CXR if indicated) - Attach Results 1. Exclude Scabies, Eczema 2. Emollient b.d. using 500 gram every 2 weeks for 8 weeks 3. Menthol 0.5% or 1% in aqueous cream (Dermacool) or Crotamiton (Eurax)	☐ ☐ ☐
Urticaria	1. Non-sedating H1-anti histamine (nsAH) for 2 weeks 2. Up-dosing nsAH up to x2 for 4 weeks (unlicensed) http://www.nice.org.uk/Advice/ESUOM31 3. Prescribe additional sedating H1-antihistamine Avoid triggers – Aspirin, NSAIDs, ACE Inhibitors Check FBC, ESR, TFTs, LFTs - Attach Results	☐ ☐ ☐